

Section 19

Substance Use, Drug Testing & the PPRCP Framework

Version 2.0 — Preliminary Draft

Petitioner: Ashley Marie Meredith, MSN | Pro Se

■ PRELIMINARY DRAFT — NOT FOR CITATION OR DISTRIBUTION WITHOUT PERMISSION

SECTION 19 — SYSTEMIC FAILURE MATRIX & CORRECTIVE ACTION SYNTHESIS

(Final Synthesis Layer — Convergent Findings Across All Domains)

Version 2.4 — June 5, 2026

19.1 PURPOSE AND FUNCTION

This section performs three functions within the petition's analytical architecture:

- (1) It synthesizes the convergent findings documented across Sections 6 through 18 into a structured matrix that maps each documented deficiency to its evidentiary source, its domain classification, its deficiency category, and its observable systemic impact.
- (2) It identifies the structural conditions that have allowed documented deficiencies to persist across multiple federal review cycles and multiple state-generated corrective action plans without remediation — establishing that the failures are systemic and institutional rather than operational and incidental.
- (3) It maps each identified deficiency cluster to the specific audit procedures that would be required to independently verify the petitioner's findings and to determine whether remediation has occurred or is underway.

The Systemic Failure Matrix (Section 13) is the central structural pillar of this petition. It is designed to function as a standalone reference document that can be transmitted to the Auditor of State, to

legislative staff, or to federal oversight bodies without requiring the recipient to reconstruct the analysis from the underlying evidentiary record.

19.2 STRUCTURAL CONDITIONS FOR PERSISTENCE

Before presenting the matrix, this section identifies four structural conditions that the evidentiary record establishes as the mechanisms by which documented deficiencies have persisted across 23 years of federal oversight:

CONDITION 1 — THE SELF-REPORTING LOOP

Every primary accountability mechanism in Iowa's child welfare oversight framework — the CFSR Statewide Assessment, the APSR, the CFSP, the CAPTA Annual Report, the CCWIS IAPD — relies on state-generated self-reporting as its primary data source. The federal oversight bodies that receive these reports do not independently verify their accuracy before acting on them. Iowa's KinderTrack data system has documented accuracy failures confirmed in official agency correspondence (A43). The new \$108.8M CCWIS system is being built on a foundation that includes this unverified data system as a planned interface (A50). The self-reporting loop is not incidental — it is architecturally embedded.

CONDITION 2 — THE CORRECTIVE ACTION INVERSION

The Performance Improvement Plan mechanism — the primary federal corrective tool applied after CFSR failures — has consistently been structured in a way that accepts performance targets below federal standards (Round 1: 25% visit rate target on a 10% baseline; Round 2: 80% target against a 95% standard), contains no consequences for missed benchmarks, and excludes domains outside DHS direct authority. Technical Bulletin #14 (December 2025) suspended this mechanism entirely, removing the only structured federal corrective response available for a state with a documented three-cycle failure record.

CONDITION 3 — THE QA PARADOX

Iowa's Quality Assurance system has been rated as a systemic strength in every CFSR round in which it was evaluated as passing (Rounds 1 and 2). Yet the same QA system produced data that was 23 points above the federal finding on caseworker visit compliance in Round 2 — demonstrating that a

system rated as adequate by its own state-administered evaluation failed its primary function. The QA Paradox documents the structural impossibility of self-audit producing reliable oversight results.

CONDITION 4 — THE INSTITUTIONAL OPACITY PATTERN

Four independent transparency failures document a pattern of structural inaccessibility of public oversight information: (a) ORR #26-927 — 60-hour fee estimate for records about a publicly-funded system, ultimately producing only materials that should be proactively available; (b) ORR #26-1536 — zero records produced; (c) HTTP 403 ACCESS DENIED on the CIO IT procurement approval process page; (d) Iowa HHS's own admission that it does not have a Data Governance Policy for a \$108.8M technology system. These four conditions together establish that the information necessary to independently verify Iowa's compliance claims is not publicly accessible.

19.3 SYSTEMIC FAILURE MATRIX

The following matrix presents the petition's convergent findings organized by deficiency category. Each row identifies a documented deficiency, its evidentiary anchors, its domain classification, and the audit procedure required for independent verification.

Column key:

Deficiency — The structural failure identified in the evidentiary record

Evidence — Primary attachment(s) and section references

Domain — Risk domain(s) per Section 5.1 framework

Def. Cat. — Deficiency category per Section 5.2 framework (F1–F13)

Audit — Procedure required for independent verification

CLUSTER A — SAFETY ASSESSMENT & CHILD PROTECTION

Deficiency: 91% safety assessment failure rate during petitioner's child removal period (Round 3, 2018–2019). Caseworker visit compliance at 29% against federal 95% standard. Pattern persists across all three completed CFSR rounds.

Evidence: A4 (Round 3 Final Report); A2 (Round 1 — 10% rate); A3 (Round 2 — 65% rate); Sections 6.3.1–6.3.3

Domain: C — Decision-Making Tools & Assessment Accuracy;

E — Outcomes, Disparities & Longitudinal Data

Def. Cat.: F1 (inadequate safety assessment); F5 (repeat findings)

Audit: Independent review of safety assessment completion rates and SDM tool implementation records for FY2018–FY2022; comparison of internal QA data to CFSR findings for same period.

Deficiency: SDM (Structured Decision Making) tool implementation failure identified as primary driver of Round 3 91% safety assessment failure rate. Tool adoption adopted without documented validation study for Iowa-specific population characteristics.

Evidence: A4; A5 (CFSR Round 4 structural analysis); Section 6.3.3

Domain: C — Decision-Making Tools

Def. Cat.: F1; F13

Audit: Review of SDM tool validation studies, implementation training records, and post-implementation outcome data for FY2016–FY2022.

Deficiency: Maltreatment in care increased 57.7% during the Round 3 monitoring period (5.21% → 8.22%). This occurred during an active Performance Improvement Plan cycle and while the state's QA system was operational.

Evidence: A4 (Round 3 Final Report, Table 1); A3 (Round 2 baseline); Section 6.3.3

Domain: B — Placement Safety & Foster Care Oversight;

E — Outcomes

Def. Cat.: F2 (placement safety); F5 (repeat findings not corrected)

Audit: Review of maltreatment in care incident records, investigation outcomes, and corrective action responses for FY2018–FY2022; comparison to CFSR reported data.

CLUSTER B — PLACEMENT SAFETY & FOSTER CARE OVERSIGHT

Deficiency: Drew household post-permanency placement safety failures.

Felony child endangerment convictions (FECR007113, FECR007114) for offenses beginning March 2021 — 15 months after finalization of TPR in December 2019.

No documented state intervention during onset period.

Evidence: A33 (criminal dockets); A34 (Gazette article); petitioner vacatur file (Buckingham affidavit — not included in petition packet); Sections 13, 14

Domain: B — Placement Safety; D — Post-Permanency Oversight

Def. Cat.: F2 (placement safety); F3 (post-permanency oversight); F12 (supervisory failure — no escalation)

Audit: Review of post-permanency monitoring records for identified placement; documentation of any safety reports received and agency response timeline; verification of placement monitoring protocol compliance.

Deficiency: Post-permanency blind spot — documented structural absence of ongoing state monitoring obligations for children in finalized adoptive or guardianship placements. No statutory or policy mechanism requires Iowa HHS to monitor child safety post-permanency in the absence of a new referral.

Evidence: A4 (Round 3); A12 (State PIP FY2026); Section 13 (PPRCP analysis)

Domain: D — Post-Permanency Placement Oversight;
G — Federal Oversight History

Def. Cat.: F3; F5; F12

Audit: Review of post-permanency monitoring policy; determination of whether any monitoring framework exists for children in placements with prior substantiated safety findings; comparison to federal child welfare policy expectations.

Deficiency: Shelby Duis (2000), Natalie Finn (2016), Sabrina Ray (2017) fatality cases. All three Ombudsman investigations document the same pattern: known risk indicators suppressed or unacted upon; contracted placement oversight failure; intake structural freeze preventing escalation.

Evidence: A28 (Shelby Duis); A29 (Natalie Finn); A30 (Sabrina Ray);
Section 12; Trinity Pattern analysis [petitioner's analytical framework — see § 12.2.6]

Domain: B — Placement Safety; F — Internal Accountability;
G — Federal Oversight

Def. Cat.: F1; F2; F6 (mandatory reporting); F12

Audit: Review of CSIU intake protocols and compliance rates for mandatory reporter documentation review; comparison of internal intake procedures to Ombudsman recommendations; determination of whether 23 Ombudsman recommendations from Shelby Duis investigation were implemented.

CLUSTER C — SUBSTANCE USE & DRUG TESTING

Deficiency: Drug testing used as sole or primary basis for Termination of

Parental Rights. Iowa HHS Communication 626 (October 2025) now explicitly prohibits this practice — establishing that the pre-2025 practice was inconsistent with current agency standards. Six-year policy-practice gap (2019–2025 codification).

Evidence: Iowa HHS Comm. 626 (available via Iowa HHS public website — not attached); A43 (Ernst letter — hair test omissions);

Section 9; petitioner's TPR order analysis

Domain: C — Decision-Making; D — Post-Permanency;

H — Data Integrity (drug testing data interface, A50)

Def. Cat.: F1 (inadequate assessment); F13 (policy-practice gap)

Audit: Review of TPR proceedings FY2016–FY2022 where drug testing was documented as primary evidentiary basis; comparison to Comm. 626 standards; review of sweat patch confirmation protocols in use during that period.

Deficiency: Methamphetamine identified as "primary reason for repeat maltreatment" in Round 1 (2003). No CFSR PIP across any of the three completed rounds contained a methamphetamine-specific intervention strategy.

The SUD service infrastructure collapsed in Round 1 and was not rebuilt.

EO-14379 (January 2026) focuses federal SUD coordination primarily on opioid-pathway MAT — no equivalent framework exists for methamphetamine.

Evidence: A2 (Round 1); A3 (Round 2); A4 (Round 3); A26 (Crossroads);

Section 6.3.1; Section 8

Domain: A — Federal Compliance; E — Outcomes; F — Workforce

Def. Cat.: F8 (service gaps); F5 (repeat findings); F13

Audit: Review of SUD treatment service availability data FY2002–present; determination of whether any Iowa-specific methamphetamine intervention framework was developed in response to CFSR findings; review of Crossroads federal SUD grant expenditure records (A26).

CLUSTER D — WORKFORCE & SERVICE INFRASTRUCTURE

Deficiency: Caseworker caseloads at 55/worker at time of Round 1 review (2003). Gains achieved between rounds (reduced to 30/worker by 2010) were reversed by budget cuts and voluntary retirements. Round 3 documents continued workforce instability. ICAPP reports (A15) document 42% collapse of child care provider infrastructure 2013–2018. No PIP targeted the

structural funding mechanism driving workforce instability.

Evidence: A2; A3; A4; A15; Section 6; Section 9

Domain: F — Workforce & Internal Accountability;

E — Outcomes (caseworker visit rates)

Def. Cat.: F9 (workforce/staffing failure); F5; F8

Audit: Review of caseworker caseload data FY2010–FY2022; determination of whether caseload reduction targets from any PIP were achieved and sustained; review of provider infrastructure funding records FY2013–FY2020.

Deficiency: 78% service referral failure rate for families accessing care (APSR FFY2026, A10). State cites service unavailability, waitlists, and transportation barriers as primary drivers. No documented state mechanism tracks whether referred services were actually received.

Evidence: A10 (APSR); A13 (CFSP); A17 (ECI Needs Assessment); Section 9

Domain: E — Outcomes; F — Workforce

Def. Cat.: F8 (service access failures); F10 (reasonable efforts gap)

Audit: Review of service referral completion data; determination of whether "reasonable efforts" findings in case records reflect actual service receipt or only referral documentation; comparison of referral rates to completion rates FY2018–FY2022.

CLUSTER E — FISCAL & PROCUREMENT ACCOUNTABILITY

Deficiency: \$36.5M GAAP violation documented in AOS reports. 60% repeat finding rate across AOS annual reports FY2009–FY2021. Contract monitoring failure identified across Medicaid MCO contracts and service provider contracts. Federal grant diversion — Crossroads SUD grant funds.

Evidence: A23; A24; A25; A26; Section 11

Domain: A — Federal Compliance; F — Fiscal Accountability

Def. Cat.: F7 (fiscal accountability); F5 (repeat findings)

Audit: Review of AOS repeat finding resolution documentation; determination of whether any corrective action plan addressed the structural conditions producing the 60% repeat finding rate; review of Crossroads federal grant expenditure records and oversight documentation.

Deficiency: \$108.8M CCWIS VISION investment approved on basis of state self-reporting containing admitted outstanding PIPs and data quality failures.

CIO approval process inaccessible. Contractor personnel not subject to documented vetting for PHI access. No Data Governance Policy exists.

Evidence: A44; A45; A46; A47; A48; A49; A50; A51; Companion Document 2; Section 18

Domain: F — Fiscal & Procurement Accountability;

H — Data Integrity & Auditability

Def. Cat.: F4 (data integrity); F5; F7; F12; F13

Audit: Review of CIO approval documentation for Google/Carahsoft contract; review of background check records for contractor personnel with PHI access; determination of whether any Data Governance Policy has been developed since the agency's April 2026 admission of its absence; independent verification of IAPD representations.

CLUSTER F — FEDERAL OVERSIGHT INTEGRITY

Deficiency: Federal CFSR methodology acknowledged by ACF (TB-14, December 2025) as potentially not accurately capturing real-world system performance. PIP accountability mechanism suspended for all states including Iowa — which has never achieved substantial conformity. Round 4 structural design relies on state self-assessment, state-controlled logistics, and optional stakeholder interviews.

Evidence: A5 (TB-14; R4D documents); Section 7; CFSR Companion Report

Domain: G — Federal Oversight History; H — Data Integrity

Def. Cat.: F4; F5; F12; F13

Audit: Review of Round 4 CFSR review findings for Iowa upon publication; determination of whether SWDI data trends documented in R4D-41 are reflected in final Round 4 findings; assessment of whether federal findings address the 91% safety assessment failure rate documented in Round 3.

Deficiency: IT jurisdictional capture — Iowa HHS IT Acceptable Use Policy (A48) subjects federal CFSR reviewers to Iowa HHS IT monitoring and audit rights during oversight reviews. The entity under review governs the data environment in which federal oversight is conducted.

Evidence: A48; A44; Section 7.3(c); Section 18.4(4); CCWIS Companion Report

Domain: G — Federal Oversight; H — Data Integrity

Def. Cat.: F4; F12

Audit: Determination of what controls govern federal reviewer data access

[illegible]

■ F4 ■ E (CCWIS), F (IT capture; KinderTrack) ■

■ F12 ■ A (QA paradox), B (post-permanency), E (CCWIS), F (IT capture)■

[illegible]

F13 (Policy-Practice Implementation Gap) is present in six of seven clusters.

These two deficiency categories are the structural connective tissue of the petition's argument: the evidentiary record does not document isolated operational failures. It documents a systemic institutional pattern in which findings are identified, corrective action is nominally initiated, and the same findings recur — across federal review cycles, across independent evaluations, across internal audit reports, and across fatality investigations spanning 23 years.

For each of the four structural conditions identified in Section 19.2, the following audit actions would be required to determine whether remediation has occurred:

Required audit action: Independent verification of at least a statistically significant sample of child welfare case outcomes against state-reported

performance data for the same period; comparison of KinderTrack data accuracy to independent case record review; assessment of whether CCWIS VISION implementation plan includes independent data validation protocols.

CONDITION 2 — CORRECTIVE ACTION INVERSION:

Required audit action: Review of all three completed PIP benchmark outcomes; determination of which benchmarks were achieved, which were missed, and what consequences (if any) were applied; assessment of the Round 3 PIP approval status and any federal action taken or not taken in response to TB-14's suspension of the PIP mechanism for Iowa.

CONDITION 3 — QA PARADOX:

Required audit action: Independent audit of Iowa's Quality Assurance system outputs for FY2018–FY2022; comparison of internal QA findings to CFSR federal findings for the same period; determination of whether the QA system has been reformed since the Round 2 23-point discrepancy finding.

CONDITION 4 — INSTITUTIONAL OPACITY PATTERN:

Required audit action: Compel production of all records responsive to ORR #26-927 and ORR #26-1536 without fee barrier; obtain and review CIO approval documentation for the Google/Carahsoft contract; obtain and review any Data Governance Policy or equivalent document developed since April 2026; review IT Acceptable Use Policy applicability to federal reviewers and assess whether that jurisdiction has ever been exercised during a CFSR review.

19.6 CONCLUSION

The evidentiary record assembled in this petition spans 23 years, nine primary federal review documents, six independent evaluations, three fatality investigations, two open records request processes, and a contemporaneous \$108.8M technology procurement. Across all of this material, two deficiency categories — F5 (repeat findings not corrected) and F13 (policy-practice implementation gap) — appear in every major evidentiary cluster.

The structural conditions that generate and sustain these deficiencies — the self-reporting loop, the corrective action inversion, the QA paradox,

and the institutional opacity pattern — are not recent developments. They are documented in the earliest materials in this record and they appear, in contemporaneous form, in the most recently produced documents.

The petitioner does not ask the Auditor of State to find that Iowa's child welfare system has failed. The federal government has already made that finding, three times, across three completed review cycles. The petitioner asks the Auditor of State to determine why the corrective mechanisms have not worked — and to exercise the independent institutional authority that Iowa Code § 11 provides to compel the production and verification of information that self-reporting and fee-gated open records processes have not made available.

The children documented in this record — Shelby Duis, Natalie Finn, Sabrina Ray, and the unnamed third fatality referenced in the 2020 Ombudsman report — are the reason this petition exists. The structural failures that are documented here are the reason it is necessary.

See also: Sections 6–18 (evidentiary basis for all matrix findings) |
Section 13 — Systemic Failure Matrix (Systemic Failure Matrix — filed as standalone exhibit) |

Section 20 (Certification of Good Faith)

End of Section 19 — Petition Body Version

Drafted June 5, 2026

19.X DOMAIN & DEFICIENCY MAPPING — SECTION 19

Domain G — Federal Oversight History & Corrective Action Response (primary):

F5 — Corrective Action Failure: Section 19 is the corrective action synthesis layer. Every remediation pathway proposed here addresses a documented F5 failure — a deficiency that prior corrective action mechanisms failed to sustain. The matrix is organized to make that failure-to-remediation chain explicit for the auditor.

All Domains A–H and all Deficiency Categories F1–F13 appear in the corrective action matrix rows. Domain and deficiency cross-referencing is by matrix row — see matrix for full mapping.

19.X AUDIT QUESTIONS GENERATED — SECTION 19

AUDIT QUESTION 19-A: This petition identifies corrective action pathways for each deficiency domain documented in Sections 6–18. What existing Iowa DHHS corrective action plans, if any, overlap with the remediation pathways identified here, and what is the implementation status of each?

AUDIT QUESTION 19-B: The Corrective Action Matrix maps deficiency categories to responsible parties and remediation timelines. What enforcement mechanism would apply if Iowa DHHS fails to implement auditor-recommended corrective actions within the specified timeframes?

AI assistance disclosure: See Section 5.8.

End of Section 19 — Version 1.0 — June 5, 2026

Audit Petition Packet v2.0 (5/2026)

AI assistance disclosure: See Section 5.8.

AI assistance was used materially in the preparation of this document. That assistance does not constitute independent verification of any factual claim.